Hordeolum, Chalazia, and Cysts

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Hordeolum

also called "stai"

- **External hordeolum**
  - **Infection of a Zeiss gland**
- **Internal hordeolum** - most of them are internal
  - **Infection of a meibomian gland** → became smaller

- ***Staphylococcus aureus* is the infectious agent in 90-95% of cases of hordeolum**
- **Abscess ("Bag of pus")**

Hordeolum : the patient will know if they have it

Subjective

- Patients usually complain of a localized painful swelling on one eyelid
- In some cases, the complaint may start as a generalized edema and erythema of the lid that later becomes localized.
- A history of similar problems is common.
- Constitutional signs and symptoms are inconsistent with a hordeolum diagnosis

doesn't have any of these

Hordeolum

Epidemiology:

Found more frequently in persons who have

- **Diabetes**
- **Other debilitating illness**
- **Chronic blepharitis**
- **Seborrhea** → greasy green
- **High serum lipids (High lipid levels increase the blockage rate of sebaceous glands, but lowering of serum lipid levels in these patients has not decreased frequency of recurrence)** fat chees and oil in food can also can cause hordeolum build up

External Hordeolum

active inflemation (HOT)

- Localized, raised, red bump near lid margin
- Tender to touch
- "Points" to skin side of lid

Tender on the touch because it is warm



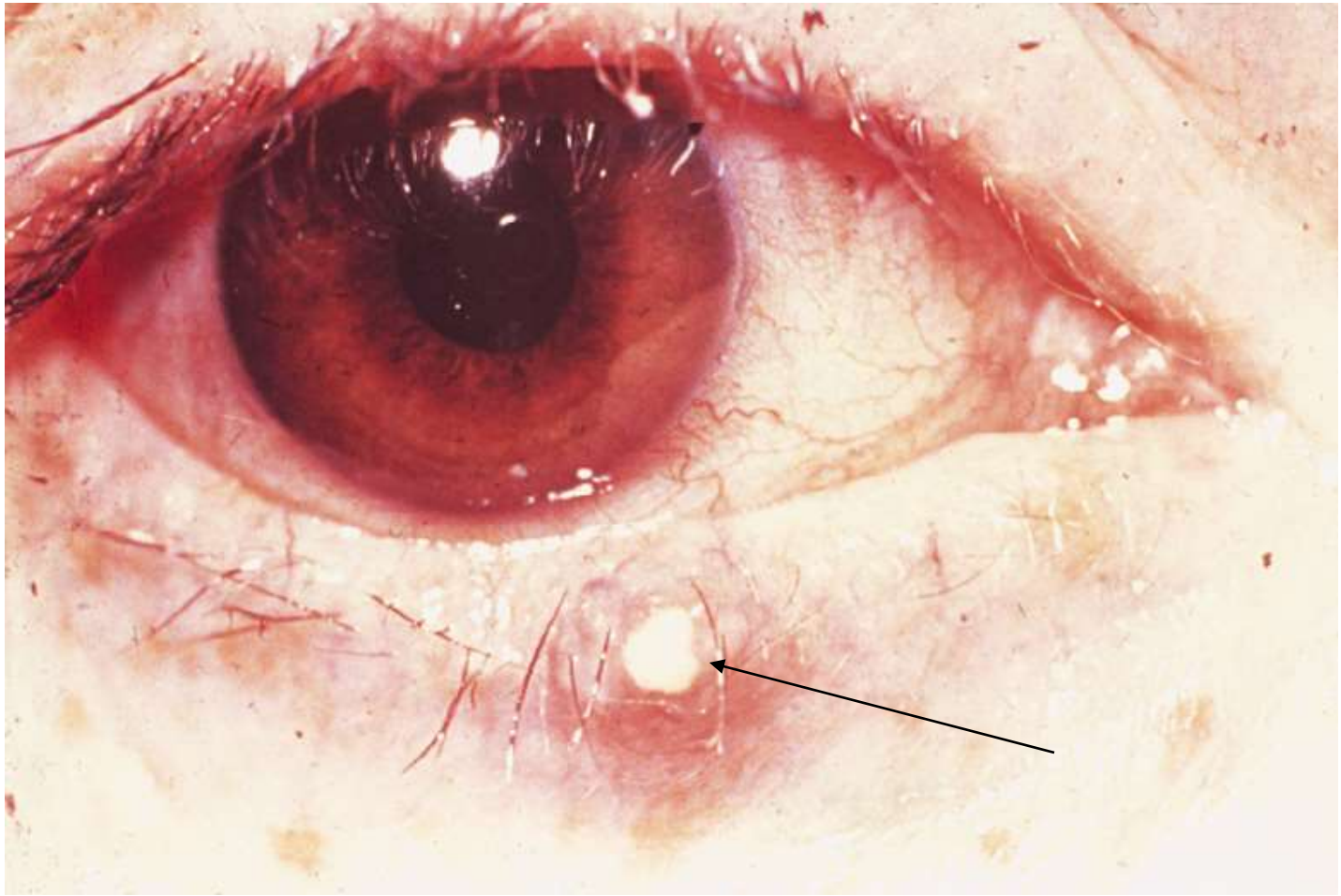
External Hordeolum

coming to the head because it is about to erupt. The skin is thinner



External Hordeolum

Hordeolum associated with staphylococcal blepharitis



External Hordeolum

- **Localized infection**

In children we see it often: child immune system is still building it, so that's why they have it so often





External Hordeolum

- No treatment; usually are self-limited
  - Most eventually point and drain by themselves
  - Discharge anteriorly, close to the lashes
- Warm soaks (qid for 15 min) are the mainstays of treatment
- Topical antibiotics may be used for recurrent lesions and for those that are actively draining
- Epilation of eyelash associated with infected follicle
  - May enhance drainage of pus



Internal Hordeolum (it is common)

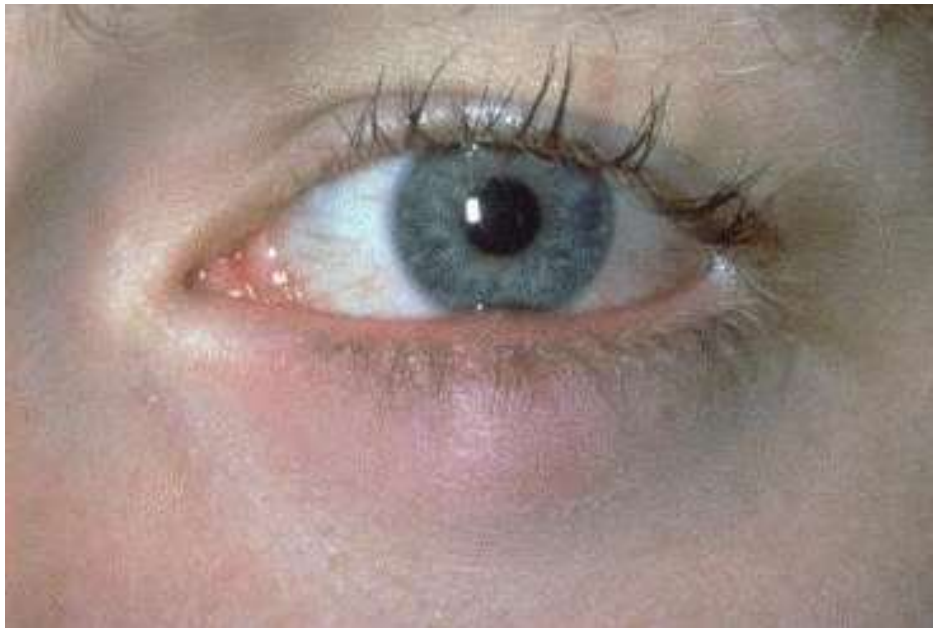
- Localized, red, raised bump
- Tender to the touch
- Points to the conjunctival side of the lid

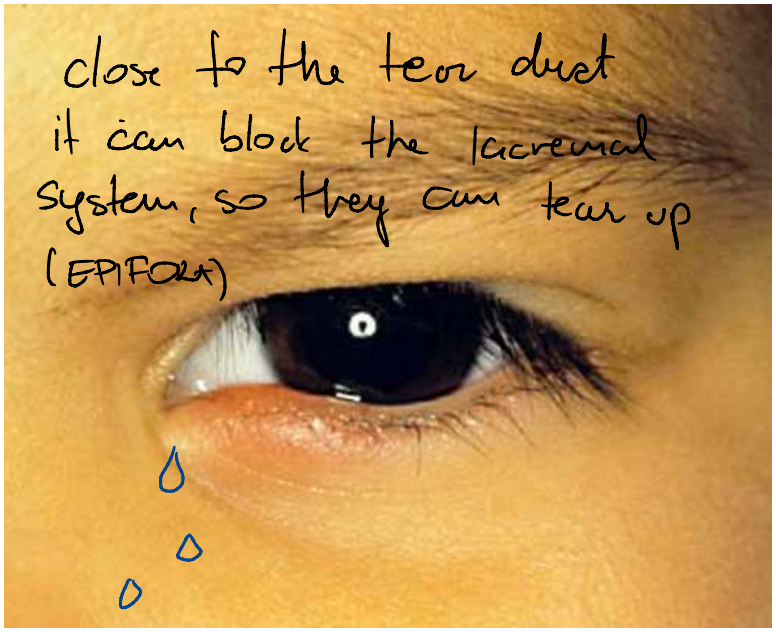


Internal Hordeolum



Internal Hordeolum



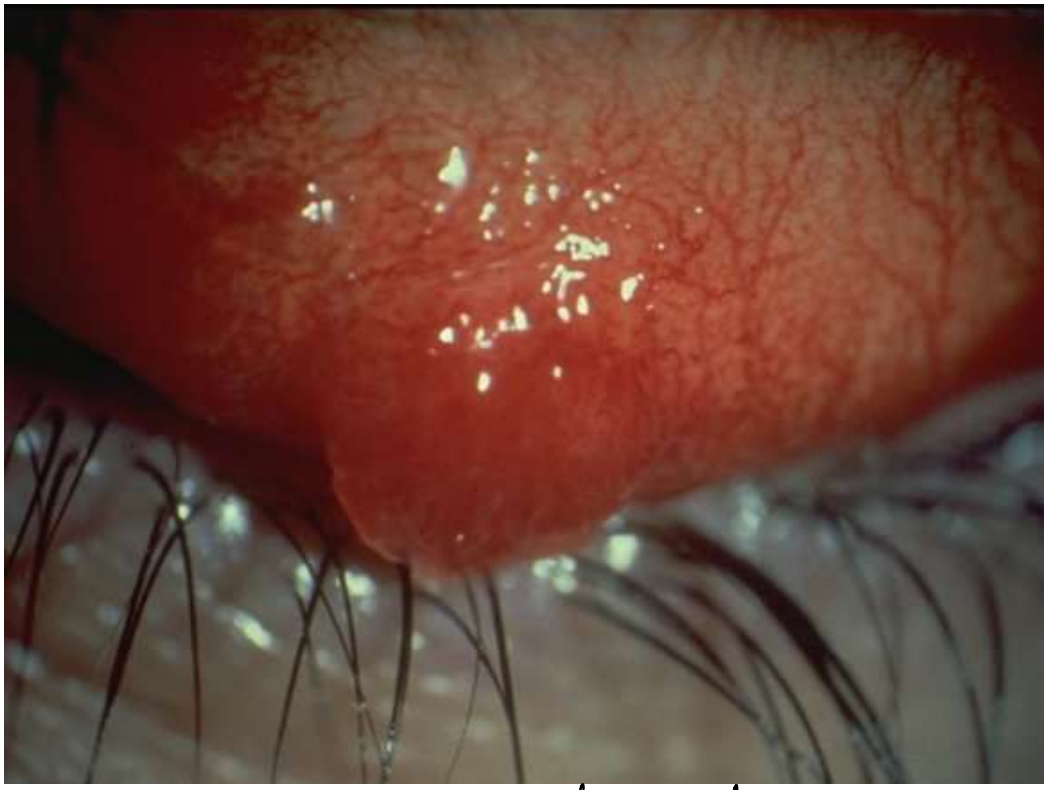


1. What bring you in 2. How long if you notice it





Internal Hordeolum



it can cause astigmatism because it pushes on cornea.





Internal Hordeolum

- **Large internal hordeolum discharging anteriorly through skin**



DO NOT SQUZZE!
harder to treat

- **Infection can spread from the gland to the lid connective tissue → preseptal cellulitis**



Internal Hordeolum

- Usually are self-limited
- Most eventually point and drain by themselves.
- Warm soaks (qid for 15 min) are the mainstays of treatment.
- Topical antibiotics may be used for recurrent lesions and for those that are actively draining
- Surgical drainage of pointed lesions can speed the healing process
- Systemic antibiotics are indicated only when inflammation has spread beyond the immediate area of the hordeolum

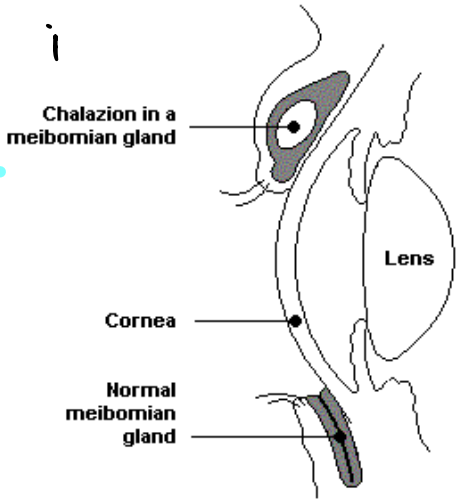
Internal Hordeolum

Drainage of a hordeolum

- Stab incision at the site of pointing using an 18-gauge needle or a #11 blade
- External incisions lead to scarring, so making external eyelid incisions or punctures is inadvisable, unless the hordeolum already is pointing externally
- A large abscess may have multiple pockets and require multiple stabs.
- Internal incisions should be made vertically to minimize the area of cornea swept by a scar during blinking
- Leave the incision open with a clean margin

Chalazion (cad)

- AKA meibomian cyst
- Lipogranulomatous infiltration of meibomian gland
- May follow an internal hordeolum
- Signs
  - Firm nodule within tarsal plate
  - Non-tender, non-inflamed
- Some refer to a internal hordeolum as an acute chalazion



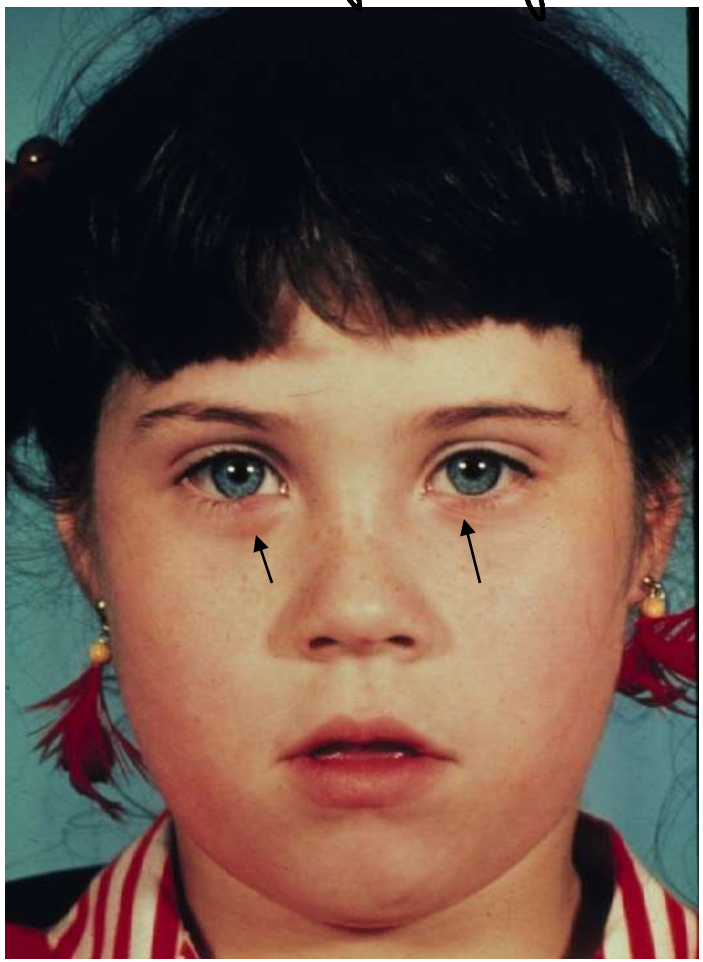
Chalazion can last for months; in child can develope:



ambliopya
astigmat.

Chalazia

it can move from eye to eye



Chalazion

it is not painful, you could flip the lid



-when it started
- does it hurt (while touching)

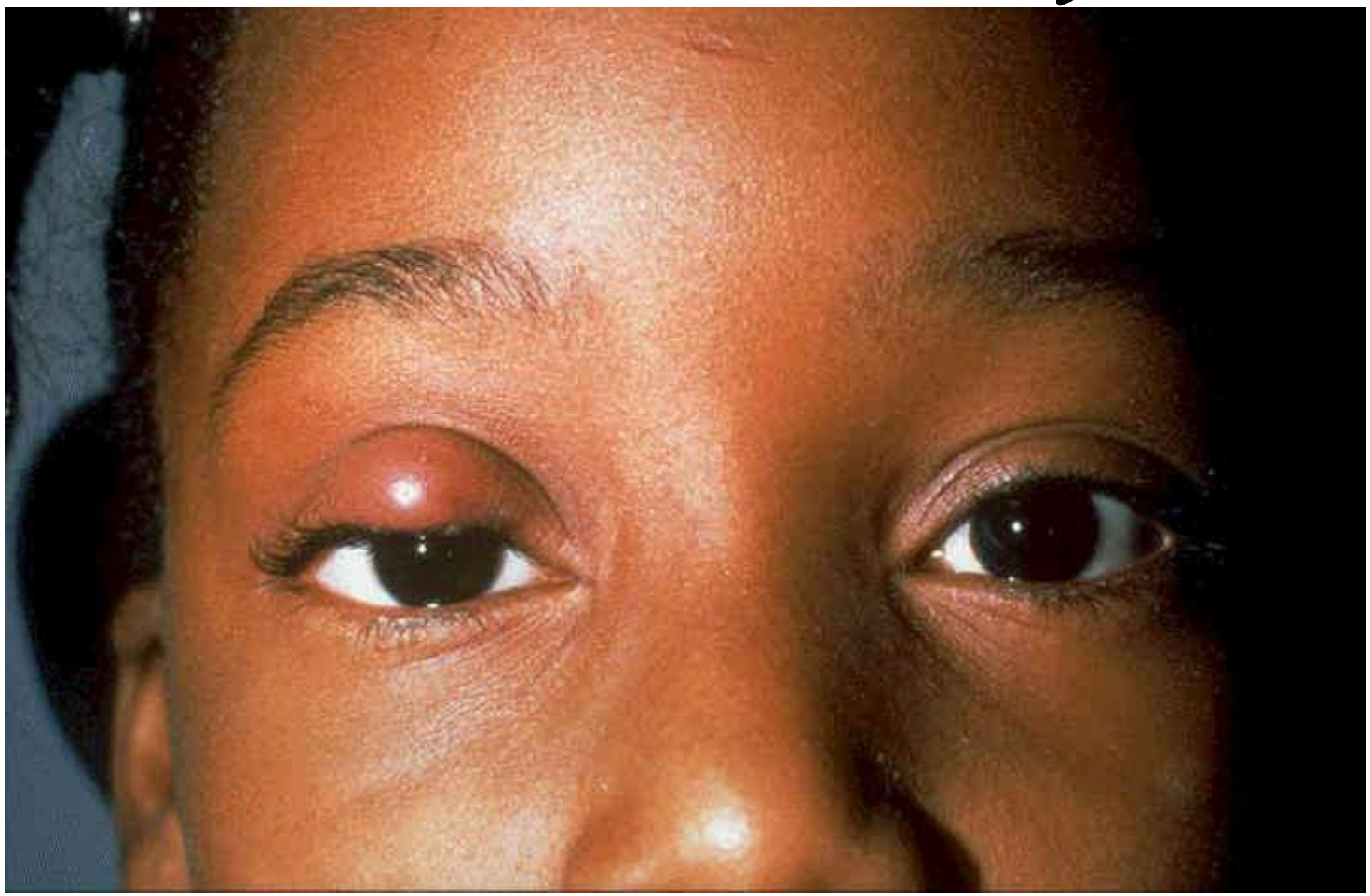
Chalazion





Chalazion

demadex can cause it, but we can't tell



Chalazion



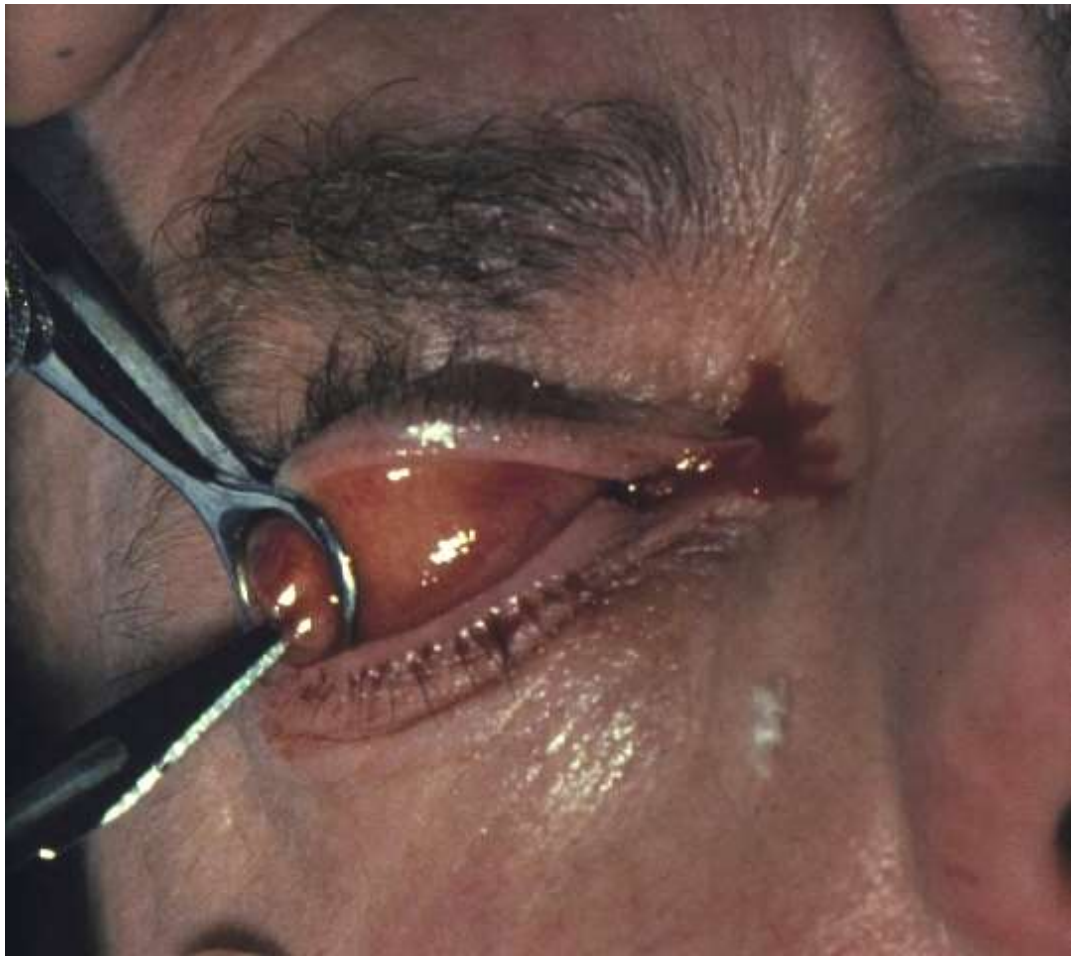
Chalazion

1. **Warm compresses**
   - 10-15 minutes
   - At least qid but q2h better
   - Work best if lesion is recent
2. **Steroid injection into lesion** you inject it directly, no anastetic
   - 0.1-0.2 ml triamcinolone injected through conjunctiva (usually 2x if not better than
3. **Incision and curettage** LAST STAGE
   - Local anesthetic
   - Chalazion clamp
   - Incision from conjunctival side
   - Curettage - scrape out material
   - Topical antibiotic ointment - X 2 days





Incision of Chalazion



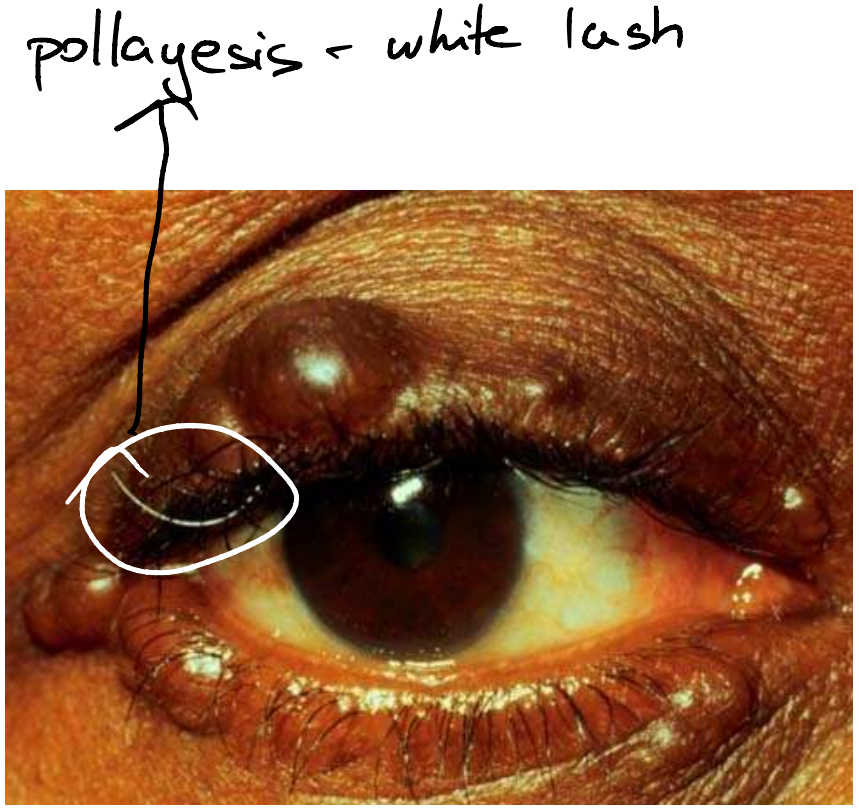
Cyst of Moll not that common

- AKA suderiferous cyst or apocrine hidrocystoma
- Arises from a blocked sweat-secreting gland associated with the cilia (gland of Moll)



Cyst of Moll

- Dome shaped
- Non-tender
- No inflammation
- Translucent
- Fluid-filled
- Anterior lid margin



Cyst of Moll

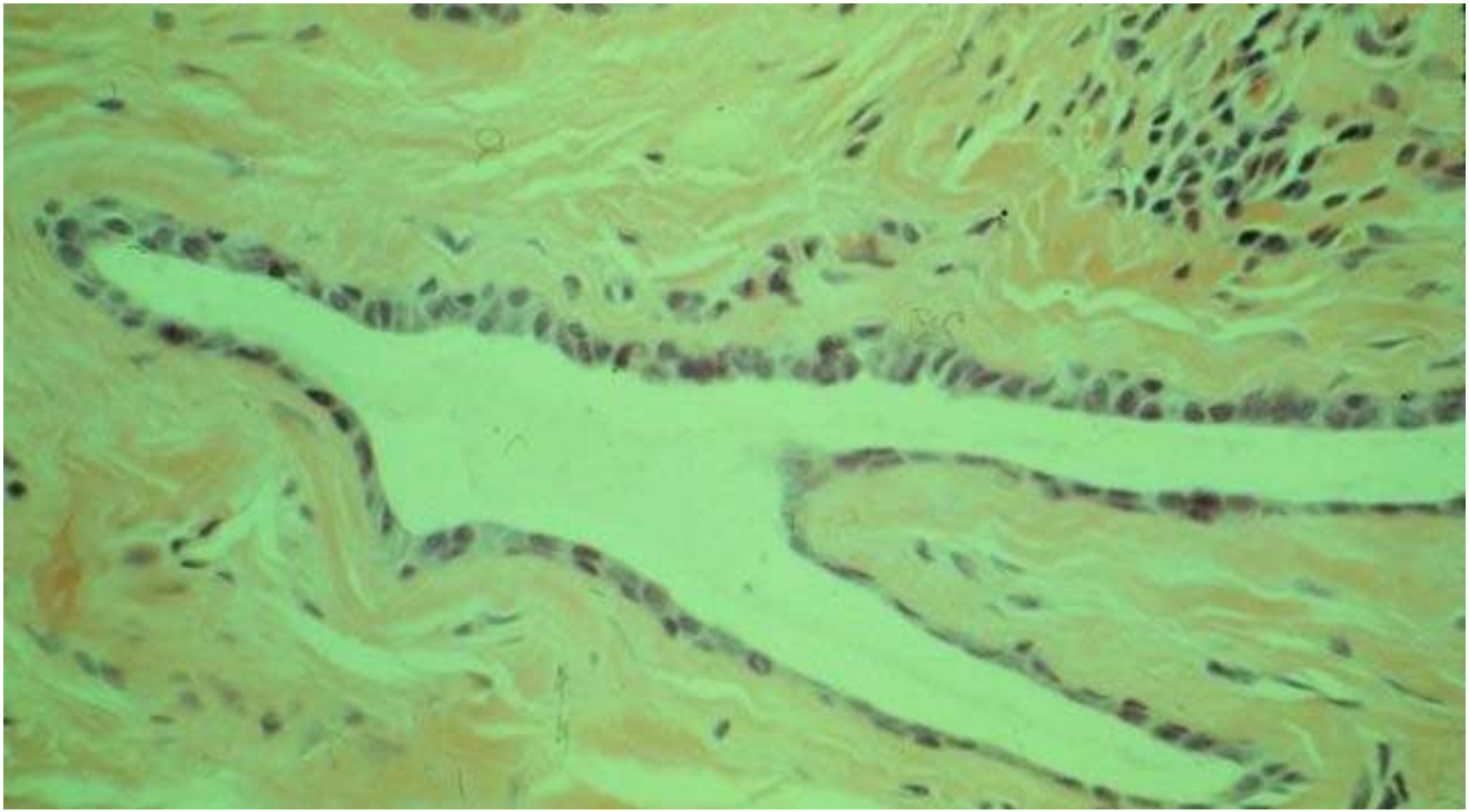


fluid filled

Image courtesy of Shaina Hewitt, PUCO Class of 2019

Cyst of Moll

histological slide of how it looks like



Cyst of Moll

Treatment

- **Primarily a cosmetic concern**
- **Can aspirate with a fine needle but recurrence is common**
- **Excision with removal of the sac to avoid recurrence**

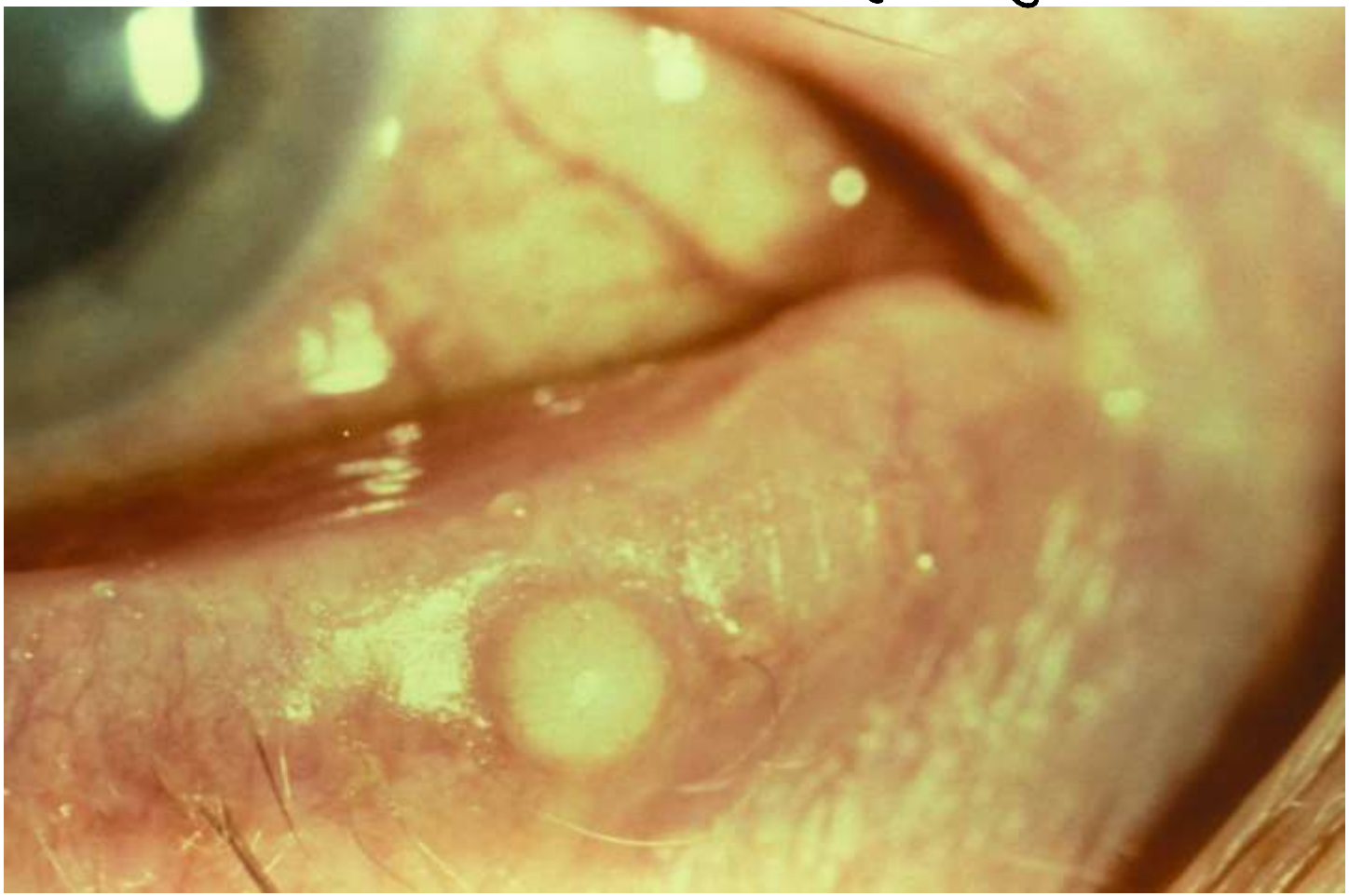


Cyst of Zeis more white than cyst of Moll

- Arises from a blocked oil-secreting gland associated with the cilia (gland of Zeis)
- Contains oily secretions and is less translucent than a cyst of Moll

Cyst of Zeis

treats like cyst of roll



Sebaceous Cyst

don't bother, but cosmeticly

- Forms when an oil gland in the skin is blocked
- Can be found on any part of the body
- Yellowish-white lumps on the skin
  - Have a central punctum
- If squeezed, exude a stringy, cheesy white material

it is more solid

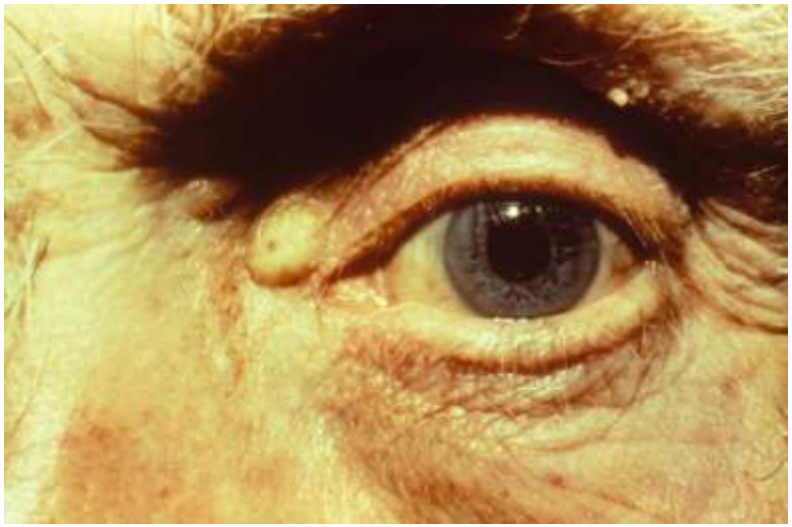


Sebaceous Cyst more localized than chalazion



Sebaceous Cyst







Differentiate sebaceous cyst from xanthelasma (aka xanthoma)…



Sebaceous cyst



Xanthelasma

Fatty deposition in both eyes and it is simetrical. If treated probably will come back

Sebaceous Cyst

- Primarily a cosmetic problem
- The total sac of the cyst must be removed
- If the cheesy material is just squeezed out, the cyst will reform
- In the case of an infected cyst, an oral antibiotic should be prescribed

Sebaceous Cyst

- **Good hygiene may prevent the formation of sebaceous cysts**
  - **Some individuals are predisposed to development of these cysts, so even careful hygiene does not prevent them**

also proper diet

THANK YOU!